

Patient ID: ParkerPeriod: April 1 to April 25, 2026Report Date: April 26, 2026Coverage: 98/585h (17%)

Patient's monitoring period was less than 30 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpmElevated HR > 95 bpmHigh HR > 100 bpm

Last 24 Hours



Vital Stat	Avg	Min	Max
Heart Rate	62 bpm	49 bpm	112 bpm
Breathing	15 brpm	10 brpm	23 brpm

Last 24 hours: one very high heart rate episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
9 AM to 10 AM	1h	1	Elevated Breathing	15	10	23	Check SpO2 and lung sounds; assess for fluid overload or early infection
9 AM to 10 AM	1h	1	Very High Heart Rate (brief)	112	112	112	Check temp, hydration, pain; review for arrhythmia or infection

Patient clinical status over 25 days from Apr 01 to Apr 25

■ HR ■ Breathing



2 episodic events Detected, spanning 2h total. Conditions: Elevated Heart Rate, Very High Heart Rate. Priority grade: Skip

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
2	2h	Very High Heart Rate	<1	17%

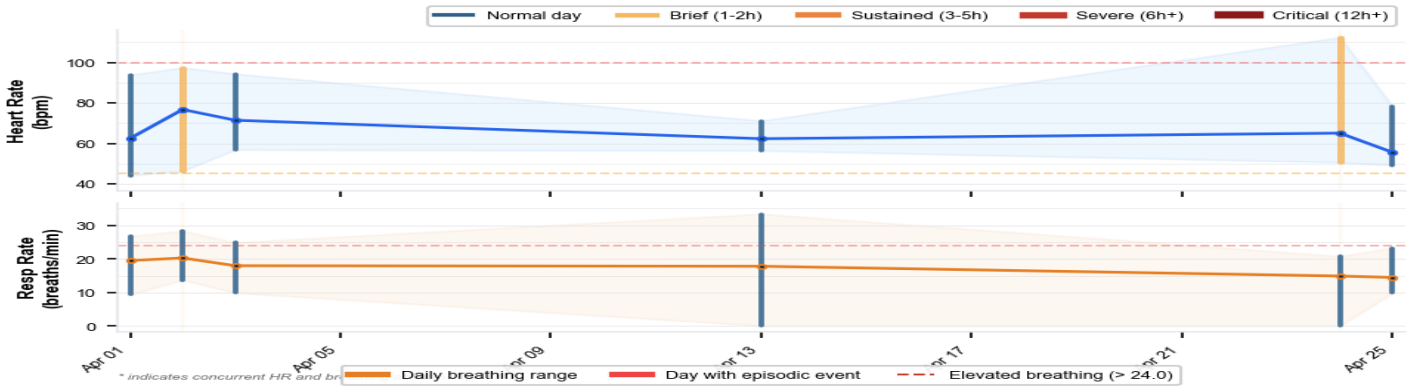
Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 24	9 AM to 10 AM	1h	1	Very High Heart Rate	112	112	112	Check temp, hydration, pain; review for arrhythmia or infection
Apr 02	1 AM to 2 AM	1h	1	Elevated Heart Rate	95	88	98	Check temp, hydration, pain; review for arrhythmia or infection

Daily recorded hours declined from 24.0 to 9.0 (62%) over the monitoring period. Insufficient data coverage (17%, below 30%).

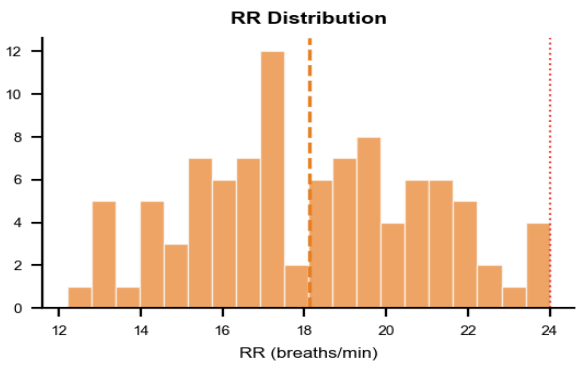
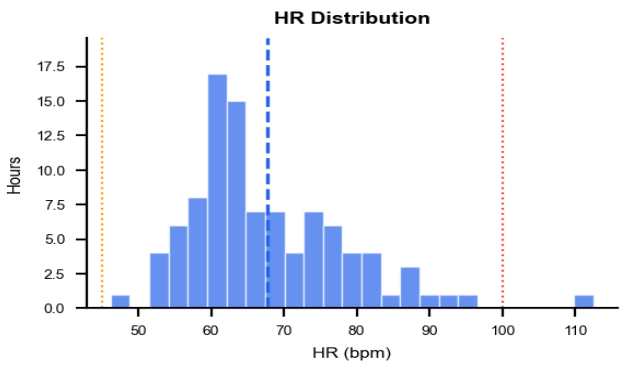
Suggested Clinical Review Actions

- Elevated Heart Rate (avg 95 bpm, peak 98 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Very High Heart Rate (avg 112 bpm, peak 112 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Overall trajectory shows 2 unstable phases with 33 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 9.0 (62%) over the monitoring period.

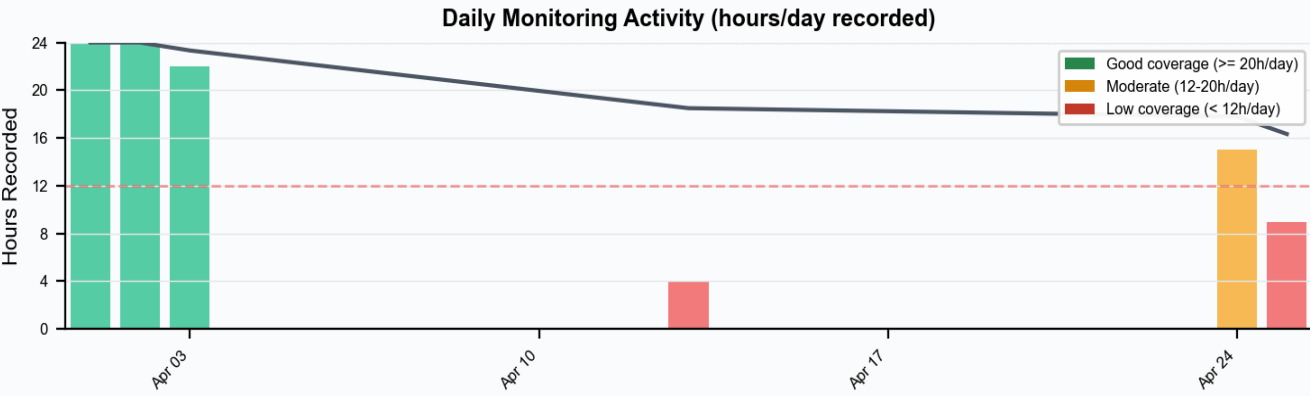


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.